

Inside the Frontier: Ownership Change and Organizational Inertia in Nursing Home Production^{*}

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Abstract

Health care providers may fail to continuously update clinical and managerial routines, leaving them inside their feasible production frontier. We study this possibility in U.S. nursing homes, where ownership changes create moments of organizational disruption and reoptimization. Using CMS administrative panel data from 2017-2024, we identify facility-level ownership transfers and estimate staffing and quality responses in a staggered-adoption event-study design. Following ownership changes, facilities reduce registered nurse and nurse aide hours, while licensed practical-nurse hours remain largely unchanged. Routine-sensitive quality measures, including catheter use, psychotropic medication use, and urinary tract infections, improve. By contrast, staffing-intensive resident outcomes such as pressure injuries, falls, weight loss, and decline in physical functioning show little evidence of broad deterioration. These patterns are consistent with ownership transitions revealing inside-frontier inefficiency: new owners do not necessarily possess superior care technology, but may be more willing to revise stale routines, reduce organizational slack, and move facilities closer to efficient operating practices. The findings suggest that governance disruptions can reshape health care production through routine replacement, with ambiguous implications for resident welfare when input reductions and process-quality improvements occur simultaneously.

Keywords: nursing homes, ownership change, staffing, difference-in-differences

JEL Codes: I11, I18, J22, L22, L33

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1 Introduction

Ownership changes are increasingly common in health care markets and have become a growing concern for policymakers, researchers, and the public. Much of this concern centers on private equity acquisitions, where outside investors may alter incentives, financial structure, and care delivery (Gupta et al., 2021). Yet private equity is only one form of ownership change. Health care providers also change ownership through acquisitions, sales, financial distress, corporate restructuring, and regulatory or administrative changes. These transitions raise a broader question: when control of an existing health care provider changes hands, how does the production of care change?

This question is distinct from the standard consolidation question emphasized in much of the health and industrial organization literature. Many mergers and acquisitions reshape market structure by changing the number or identity of competing providers in a local market. In those settings, the central concerns are often market power, prices, wages, bargaining, and entry. The ownership changes we study in this paper are different. They involve transfers of control over existing nursing homes. The facility remains in the same location, serves the same local population, and faces the same local market environment. If these ownership changes affect care, the effects are unlikely to operate through mechanical changes in market structure. Instead, they operate through internal production decisions: staffing, task allocation, clinical routines, monitoring, documentation, medication management, and managerial oversight.

The expected effect of ownership change is ambiguous. A distressed facility may be acquired by an owner seeking to stabilize by cutting costs. A profitable facility may be purchased because a buyer expects to preserve the existing operating model and extract returns. An independent owner may sell because of retirement, limited access to capital, or limited willingness to invest in updated managerial and clinical routines. In other cases, a buyer may believe that the facility is operating below its potential and can be improved through changes in staffing policies, monitoring systems, medication-management practices,

or care protocols. Ownership change may therefore lead to quality-reducing cost cutting, little operational change, or efficiency-enhancing reorganization. This ambiguity is central: the same type of transition can reflect different pre-existing conditions and can produce different production responses.

We study ownership change as a potential moment of organizational disruption and reoptimization. Health care providers may not continuously update clinical and managerial routines, even when better practices are feasible. Routines can persist because they are familiar, because managers face limited attention, because staff resist change, or because incumbent owners lack the incentive or capacity to reorganize production. In this sense, facilities may operate inside their feasible production frontier: they may use more inputs than necessary for a given level of measured quality, or produce lower quality than is feasible with their existing inputs. A change in ownership can disrupt these routines. New owners do not necessarily possess a superior care technology, but they may be more willing to revise old routines, alter staffing composition, standardize monitoring, or implement labor-saving care processes.

Nursing homes are an especially useful setting in which to study these mechanisms. Residents are medically vulnerable and often rely on staff for activities of daily living. At the same time, nursing homes operate in a heavily regulated environment where short-run prices are largely fixed by public reimbursement and where staffing is a central input into care production. Because labor is both costly and directly tied to care delivery, staffing levels, staffing composition, and care routines are natural margins through which new owners may alter production. Changes in nurse staffing are therefore important not only as labor-market outcomes, but also because they may affect resident welfare.

A large body of evidence links nursing home staffing to quality. Registered nurses, licensed practical nurses, and certified nursing assistants perform different roles in the production of care. Registered nurses provide higher-skill clinical assessment, supervision, care planning, and medication-related oversight. Licensed practical nurses provide routine clin-

ical services and medication-management tasks. Certified nursing assistants provide much of the hands-on daily care, including assistance with bathing, dressing, toileting, feeding, and mobility (Lin, 2014). Because these staff types differ in training, scope of practice, and cost, ownership changes may affect not only total staffing hours but also the skill mix of care. Prior research shows that staffing levels are associated with regulatory deficiencies and quality outcomes, but staffing is also an endogenous managerial choice made jointly with budgets, resident acuity, regulatory pressure, and other inputs (Harrington et al., 2000; Lin, 2014).

The broader nursing home literature also shows that ownership incentives and financial constraints can shape care quality. Private equity ownership has received particular attention because it may introduce stronger financial incentives, new debt structures, or more aggressive cost-control strategies. Gupta et al. (2021) find that private equity acquisition of nursing homes is associated with worse patient outcomes and changes in staffing composition. Other work emphasizes the importance of reimbursement, Medicaid dependence, public reporting, and regulation in shaping staffing and quality choices (Park and Werner, 2011; Bowblis and Brunt, 2025; Bowblis and Ghattas, 2017). These studies suggest that ownership and financial incentives matter, but they also point to a broader issue: changes in control can alter the objectives and constraints faced by the facility, potentially reshaping internal production decisions.

Ownership change may also affect care production through channels that are not captured by a simple cost-cutting story. Facilities may fail to continuously update clinical and managerial routines. Existing staffing patterns, medication-management practices, monitoring systems, and documentation processes can persist because they are familiar, because incumbent managers have limited attention, or because owners lack the incentive or capacity to reorganize production. In this sense, some facilities may operate inside their feasible production frontier. They may use more inputs than necessary to produce a given level of measured quality, or they may produce lower measured quality than is feasible with existing

resources. An ownership transition can disrupt these routines. New owners do not necessarily possess a superior care technology, but they may be more willing to revise old practices, standardize monitoring, alter staffing composition, or implement labor-saving routines.

2 Data and Methods

2.1 Data Sources

We utilize three main sources of data: Nursing Home Compare (NHC) Archive data, Healthcare Cost Report Information System (HCRIS), and Payroll Based Journal (PBJ) data. The NHC Archive contains monthly facility level information on ownership, chain affiliation, certification status, as well as quarterly reported quality measures. Second, we use HCRIS, also known as the Medicare Cost Reports, which contains annual cost reports submitted by every Medicare certified nursing home in the United States. The cost reports contain information about costs, revenue, financial data for each reporting facility, and dates of ownership changes. Lastly, we use the PBJ data to obtain staffing information. The PBJ data contains the resident census and the number of hours worked by various types of NH staff on a daily basis.

2.2 Sample selection

We utilize two samples in our study, the first is our staffing panel which is constructed at the facility-month level and the second is our quality metric panel which is constructed at the facility-quarter level. We construct both samples using the following criteria. First, we limit our samples to the 48 contiguous U.S. states from the period January 2017 to June 2024. Secondly, the NHC Archive data is matched to the HCRIS data on a unique nursing home provider number. Because HCRIS only contains information on freestanding nursing homes, our samples exclude hospital-based nursing homes. Hospital based nursing homes differ from freestanding facilities in their structure and financing. Because they are within

hospitals, staffing policies and resource allocation may be jointly determined with hospital operations and respond to hospital shocks and regulations rather than nursing home specific incentives. We therefore exclude hospital based facilities to maintain a more homogeneous set of providers and to ensure ownership changes reflect changes in nursing home management rather than broader hospital system changes. Hospital based nursing homes make up about 12 percent of all nursing homes in the United States.

2.3 Identifying Ownership Changes

No single data source provides a comprehensive record of ownership changes for U.S. nursing homes, so we combine information from HCRIS and the NHC Archive in a two-step identification and verification process to identify ownership changes.

First, we use the HCRIS data which contains an indicator for an ownership change, and corresponding date, in each cost report after an ownership change. We use this as our flag for ownership changes during the study period. However, these indicators may capture administrative or legal changes that do not reflect meaningful transfers of control. Thus, to verify these changes, our second step was to validate each potential ownership change in the HCRIS data with the NHC Archive data. The NHC Archive data reports ownership shares for each owner associated with a facility on a monthly basis. One issue with the NHC Archive data is that it reports the direct owners and indirect owners of each nursing home. For example, a limited liability corporation may directly own the nursing home, but that limited liability corporation is owned by different individuals. Therefore, we identified ownership changes to occur among direct and indirect owners using the following criteria:

Using this information, we define a change in ownership when a majority of ownership shares turn over between month $t - 1$ and month t . Intuitively, this captures transactions in which control of the facility changes hands, including cases where a new operator acquires most or all shares. Secondly, when ownership percentages are partially or completely missing, we approximate shares by normalizing any reported percentages and, where necessary,

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assigning equal weights across remaining owners. To reduce false positives from purely legal restructuring, we do not flag a change in ownership if at least 80 percent of the ownership stake remains within the same surname family (e.g., a change from “Smith Holdings LLC” to “Smith Family Partners LP”).

Using these verified ownership changes, the analytic sample included nursing homes that either (i) have no ownership change in either source or (ii) have exactly one ownership change in each source with the ownership change date within six months of one another.

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For these Nursing Homes, we define the change in ownership month as the first month in which HCRIS identifies a change. All dates are converted to calendar months, so changes occurring at the beginning or end of a month are treated identically.

2.4 Nursing Staff Levels

The nursing staff types we use in our study are registered nurses (RNs), licensed practical nurses (LPNs), and certified nursing assistants (CNAs). RNs are the most highly trained nursing staff and are responsible for assessment, care planning, medication administration, and supervision of other nursing personnel. LPNs provide basic medical care under the supervision of RNs and physicians, including monitoring vital signs and administering certain treatments. CNAs provide the majority of direct daily care to residents, assisting with activities of daily living. Because these staff types differ in training, scope of practice, and compensation, changes in staffing composition may reflect different organizational responses to ownership changes. We therefore examine each staff category separately, as well as total nurse staffing.

Nursing staff levels are calculated from PBJ data, which report daily staffing hours and resident census. For each facility i , calendar month t , and staff type, we aggregate daily hours and resident days to construct hours per resident day (HPRD):

$$\text{HPRD}_{it} = \frac{\sum_{d \in t} H_{id}}{\sum_{d \in t} R_{id}}, \quad (1)$$

where H_{id} is the number of hours worked by the staff type in facility i on day d , and R_{id} is the number of resident days in facility i on day d . We construct separate HPRD measures for RNs, LPNs, and CNAs, and define total HPRD as the sum of the three type-specific measures. For quality specifications estimated at the facility-quarter level, we construct quarterly staffing measures analogously by summing hours and resident days over all days in the quarter before taking the ratio.

We also construct additional measures of data quality. First, we calculate a monthly PBJ coverage measure, defined as the share of days in the month with complete staffing records. This measure allows us to identify incomplete data in our sample and describe our data completeness. We use this measure for sample construction and descriptive purposes, excluding facilities and observations with low reporting coverage. Secondly, we track reporting gaps, defined as the number of months since a facility's previous PBJ observation, and use this variable in robustness checks that restrict the sample to facilities with more continuous reporting.

Finally, we exclude a small number of observations with implausible staffing levels: months in which RN and LPN HPRD are both zero, total HPRD is below 1.5 or above 12, or CNA HPRD exceeds 5.25. These observations are outliers that likely reflect reporting errors rather than genuine variation in staffing. These exclusion criteria align with the criteria outlined in the CMS NHC Technical Users Guide.

2.5 Quality Metrics

We measure quality using selected CMS quality measures from the NHC Archive. These measures are reported at the facility-quarter level and are constructed from resident as-

This entire section is new

assessments submitted through the Minimum Data Set (MDS). We focus on measures that are plausibly responsive to ownership-related changes in staffing, care routines, medication management, monitoring, and documentation. For each measure used in the analysis, lower values indicate better measured quality.

We group the selected measures into two categories. The first category consists of routine-sensitive process measures: catheter use, anti-psychotic medication use, and anti-anxiety or hypnotic medication use. Catheter use measures the percentage of long-stay residents with a catheter inserted and left in their bladder. Catheters may be clinically appropriate in some cases, such as urinary retention, selected wound-care situations, or end-of-life care, but unnecessary catheter use can increase infection risk and may reflect facility routines around toileting assistance and monitoring. Anti-psychotic medication use measures the percentage of long-stay residents who received an anti-psychotic medication and anti-anxiety or hypnotic medication use measures the percentage of long-stay residents who received an anti-anxiety or hypnotic medication. Both of these metrics capture medication-management practices among long-stay residents. These medications may be clinically appropriate for some residents, but their use can also reflect prescribing routines, behavioral-management practices, staff monitoring, and reliance on pharmacological approaches to resident care.

The second category consists of resident outcome measures: pressure injuries, falls with major injury, weight loss, decline in physical functioning, and urinary tract infections. These measures are more directly related to resident health and functioning and may be affected by staffing-intensive care processes. Pressure injuries may reflect repositioning, skin checks, hygiene, nutrition, and monitoring. Falls with major injury capture serious resident safety events that may be affected by supervision, mobility assistance, medication management, and environmental safety. Weight loss captures clinically meaningful unintended weight loss¹ and may reflect nutrition, feeding assistance, monitoring, or changes in resident condition. Decline in physical functioning captures worsening dependence in mobility or activities

¹This does not include residents who are on a physician-prescribed weight-loss plan.

of daily living. Urinary tract infections may reflect resident frailty and clinical need, but also catheter use, hydration, toileting assistance, hygiene, monitoring, and documentation practices.

2.6 Other Controls

To account for facility characteristics that may be correlated with staffing, quality, and ownership changes, we included a number of control variables that are consistent with other studies (cite here). The following variables come from NHC and HCRIS and are used as controls in all specifications: Ownership type (government, non-profit, and for-profit), chain affiliation, total number of beds, occupancy rate, payer mix (percent of revenue coming from Medicare, and percent of revenue coming from Medicaid), and acuity. Our measure of acuity is case-mix provided by CMS in the NHC archive data. During our study period CMS altered the methodology used to construct case-mix indices. As a result, raw case-mix values are not directly comparable over time. To address this issue, we measure acuity using relative rankings rather than levels. Specifically, we construct case-mix quartiles within each state and calendar month.

2.7 Analytic Approach

Ownership changes occur at different dates across facilities during the study period. Because these changes are not randomly assigned, our empirical strategy does not compare facilities that change ownership to facilities that do not in levels. Instead, we exploit the timing of verified control-changing ownership transitions to compare changes within a facility before and after ownership change, relative to contemporaneous changes among facilities that have not yet experienced a change or never experience one during the study period. All specifications include facility fixed effects, time fixed effects, and observable time-varying facility characteristics.

This staggered timing raises a standard concern for two-way fixed effects designs. In

staggered-adoption settings, conventional two-way fixed effects estimates can be sensitive to treatment-effect heterogeneity and comparisons involving already-treated units (Goodman-Bacon, 2021). We therefore emphasize event-study estimates using alternative comparison groups. This design is well suited to the institutional setting. Following an ownership change, the facility remains in the same location, operates under the same certification framework, and serves the same local market. As a result, any post-transition changes are likely to operate through internal production decisions, including staffing, routines, and management practices, rather than through a mechanical change in local market structure.

A complication in defining event time is that the recorded ownership change month may not coincide with the beginning of the transfer process. The ownership change date we observe in HCRIS is the effective date of the change, or the closing date. However, the sale process begins earlier, when the buyer and seller negotiate and sign a contract. Ideally, we would observe the contract signing date or the month in which transition planning begins. Because these dates are not observed, the months immediately preceding the recorded closing date may already reflect operational responses to the pending transfer. During this pre-transfer period, incumbent managers or prospective owners may adjust staffing policies, scheduling, hiring, agency use, or other operating routines. Because staffing can adjust relatively quickly, these months may not represent untreated facility behavior. For this reason, the preferred staffing event-study specification excludes $\tau = -3, -2, -1$ and uses $\tau = -4$ as the reference period. For quality outcomes, which are measured quarterly, the timing concern is different. The ownership-change quarter, $\tau = 0$, may contain care, assessment, and documentation from both before and after the transfer. We therefore exclude $\tau = 0$ and use $\tau = -1$ as the reference period.

We estimate the following set of event-study regressions:

$$S_{im} = \sum_{\tau=-24, \tau \neq -4, -3, -2, -1}^{24} \beta_{\tau} \mathbf{1}(T_{im} = \tau) + X_{im}\gamma + \mu_i + \lambda_m + \varepsilon_{im}. \quad (2)$$

For staffing outcomes, S_{im} denotes RN, LPN, CNA, or total HPRD for facility i in month

I originally only introduced this after the specifications, but I moved it earlier this time

m . The variables $\mathbf{1}(T_{im} = \tau)$ are event-time indicators for whether facility i is τ months from ownership change. The coefficients β_τ trace monthly staffing dynamics relative to four months before the ownership change. The months $\tau = -3, -2, -1$ are excluded to remove the period most likely to be influenced by the pre-transfer adjustment window.

$$Q_{iq} = \sum_{\tau=-8, \tau \neq -1, 0}^8 \theta_\tau \mathbf{1}(T_{iq} = \tau) + X_{iq}\pi + \mu_i + \lambda_q + \nu_{iq}. \quad (3)$$

For quality outcomes, Q_{iq} denotes a selected CMS quality measure for facility i in quarter q . The variables $\mathbf{1}(T_{iq} = \tau)$ are event-time indicators for whether facility i is τ quarters from ownership change. The coefficients θ_τ trace quarterly quality dynamics relative to the quarter before ownership change. The ownership-change quarter, $\tau = 0$, is excluded because it may contain both pre- and post-transfer care, documentation, and reporting.

In both equations, X_{im} and X_{iq} denote vectors of facility controls including: ownership type, chain affiliation, beds, occupancy rate, payer mix, and state-period case-mix quartile indicators. Facility fixed effects μ_i absorb time-invariant differences across nursing homes, while month fixed effects λ_m and quarter fixed effects λ_q absorb common shocks over time. Standard errors are two-way clustered by facility and calendar period.

For quality outcomes, we estimate specifications both with and without RN, LPN, and CNA HPRD controls. Because staffing is itself affected by ownership change, the staffing-controlled specifications are not interpreted as preferred estimates of the total effect of ownership change on quality. Instead, they are used descriptively to assess whether the estimated quality responses are attenuated after conditioning on measured staffing inputs.

We also estimate average post-change specifications that replace the event-time indicators with a single post-ownership-change indicator:

$$Z_{ip} = \delta Post_{ip} + X_{ip}\gamma + \mu_i + \lambda_p + \varepsilon_{ip}. \quad (4)$$

In equation (4) $p = m$ for staffing outcomes and $p = q$ for quality outcomes. Z_{ip} denotes

either a staffing outcome in facility-month im or a quality outcome in facility-quarter iq . The indicator $Post_{ip}$ equals one after the ownership change and zero otherwise. The coefficient δ summarizes the average post-ownership-change difference in the outcome, relative to the pre-change period, after accounting for facility fixed effects, time fixed effects, and time-varying controls. For staffing outcomes, the preferred post specifications are estimated after excluding the same immediate pre-transfer months used in the event-study specification. For quality outcomes, the preferred post specifications exclude the ownership-change quarter. These post specifications are used to summarize the average post-change difference and should be interpreted alongside the event-study estimates.

For staffing outcomes, we estimate event study and conventional difference-in-differences in both levels and logs. Log specifications allow coefficients to be interpreted as approximate percentage changes and reduce sensitivity to differences in baseline staffing levels across facilities. Log specifications are omitted when staffing is zero.

2.7.1 Parallel Trends

The identifying assumption of equations 2 and 3 is, absent an ownership change, treated facilities would have followed similar staffing and quality trends as not-yet-treated and untreated facilities. We assess this assumption using the pre-treatment coefficients in the event-study models and joint Wald tests of whether these coefficients are equal to zero.

For staffing outcomes, we compare pre-trend tests from the full pre-window to those from the preferred donut specification. The full pre-window includes the months immediately preceding the recorded ownership change and uses $\tau = -1$ as the reference period. The donut specification excludes $\tau \in \{-3, -2, -1\}$ and uses $\tau = -4$ as the reference period. Table 1 reports the results of these Wald tests. The improvement in the pre-trend tests after removing the immediate pre-transition months supports the interpretation that those months are influenced by the pre-transfer adjustment window.

Table 1: Joint Wald Tests of Pre-trends for Monthly Staffing

	Outcomes			
	RN	LPN	CNA	Total
Panel A: HPRD				
2 Year Full Pre-Window	93.04 (23) [0.0000]	39.36 (23) [0.0181]	36.25 (23) [0.0389]	48.73 (23) [0.0013]
2 Year Window with Donut	27.52 (20) [0.1213]	34.10 (20) [0.0254]	11.36 (20) [0.9362]	23.90 (20) [0.2466]
1 Year Window with Donut	11.23 (8) [0.1892]	6.39 (8) [0.6039]	1.43 (8) [0.9938]	3.73 (8) [0.8806]
Panel B: Log(HPRD)				
2 Year Full Pre-Window	47.06 (23) [0.0022]	38.05 (23) [0.0252]	35.80 (23) [0.0433]	44.39 (23) [0.0047]
2 Year Window with Donut	26.11 (20) [0.1622]	37.18 (20) [0.0111]	10.86 (20) [0.9498]	22.71 (20) [0.3033]
1 Year Window with Donut	4.30 (8) [0.8294]	6.81 (8) [0.5574]	1.74 (8) [0.9879]	4.45 (8) [0.8140]

Notes: Each cell reports the Wald χ^2 statistic for the joint null that all pre-treatment event-time coefficients equal zero, followed by degrees of freedom in parentheses and the p-value in brackets.

Tested windows and reference periods: 2 Year Full Pre-Window tests $\tau = -24$ to $\tau = -2$ with reference $\tau = -1$; 2 Year Window with Donut tests $\tau = -24$ to $\tau = -5$ with reference $\tau = -4$ (dropping $\tau = -3, -2, -1$); 1 Year Window with Donut tests $\tau = -12$ to $\tau = -5$ with reference $\tau = -4$ (dropping $\tau = -3, -2, -1$).

Sample sizes (rows): 2 Year Full Pre-Window ($N = 630, 420$); 2 Year Window with Donut ($N = 626, 311$); 1 Year Window with Donut ($N = 626, 311$).

All specifications include facility and month fixed effects and covariates: *government*, *non-profit*, *chain*, *beds*, *occupancy rate*, *percent Medicare*, *percent Medicaid*, and state case-mix quartile indicators.

For quality outcomes, we conduct analogous Wald tests using the quarterly event-study specification. The full pre-window tests whether the coefficients for quarters $\tau = -8$ through $\tau = -1$ are jointly equal to zero. Our preferred quality specification excludes the ownership-change quarter, $\tau = 0$, because this quarter may combine pre- and post-transfer care, assessment, and documentation. Table 2 reports the results of these Wald tests.

Table 2: Joint Wald Tests of Pre-Trends for Quarterly Quality Measures

Outcome	2 Year Full Pre-Window	2 Year, with Donut	1 Year, with Donut
Catheter	12.10 (7) [0.0972]	13.00 (7) [0.0721]	2.01 (3) [0.5707]
Antipsychotic	7.49 (7) [0.3793]	7.89 (7) [0.3425]	3.43 (3) [0.3301]
Hypnotics	3.29 (7) [0.8570]	3.33 (7) [0.8532]	0.53 (3) [0.9117]
Pressure injuries	12.76 (7) [0.0781]	12.62 (7) [0.0820]	1.77 (3) [0.6211]
Falls	4.13 (7) [0.7649]	3.88 (7) [0.7937]	0.24 (3) [0.9711]
Weight loss	8.09 (7) [0.3251]	7.82 (7) [0.3489]	3.60 (3) [0.3083]
ADL increase	13.10 (7) [0.0697]	12.68 (7) [0.0802]	2.21 (3) [0.5306]
UTI	5.98 (7) [0.5422]	5.95 (7) [0.5450]	0.97 (3) [0.8081]

Notes: Each cell reports the Wald χ^2 statistic for the joint null that all pre-treatment event-time coefficients equal zero, followed by degrees of freedom in parentheses and the p-value in brackets.

The 2 Year Full Pre-Window tests $\tau = -8$ through $\tau = -1$, with $\tau = -1$ as the omitted reference period.

The ownership-change-quarter-excluded specifications omit $\tau = 0$ because the quarter of ownership change may combine pre- and post-transfer care, assessment, and documentation.

The 1 Year Window tests $\tau = -4$ through $\tau = -1$, with $\tau = -1$ as the omitted reference period.

Sample sizes by outcome: 2 Year Full Pre-Window [Catheter=188,762; Antipsychotic=201,403; Hypnotics=150,667; Pressure injuries=140,715; Falls=197,246; Weight loss=198,690; ADL increase=200,578; UTI=196,502].

Sample sizes by outcome: 2 Year, with Donut [Catheter=187,542; Antipsychotic=200,132; Hypnotics=149,711; Pressure injuries=139,764; Falls=195,998; Weight loss=197,438; ADL increase=199,311; UTI=195,249].

Sample sizes by outcome: 1 Year, with Donut [Catheter=187,542; Antipsychotic=200,132; Hypnotics=149,711; Pressure injuries=139,764; Falls=195,998; Weight loss=197,438; ADL increase=199,311; UTI=195,249].

All specifications include facility and quarter fixed effects and covariates: government + non_profit + chain + beds + occupancy_rate + pct_medicare + pct_medicaid + cm_q_state_2 + cm_q_state_3 + cm_q_state_4.

2.7.2 Additional Analyses

3 Results

3.1 Summary Statistics

Table 3 reports summary statistics for the monthly staffing sample. The final sample consists of approximately 7,806 unique skilled nursing facilities and 632,231 facility-month observations spanning January 2017 through June 2024. Of these facilities, 1589 experience an ownership change during the sample period.

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Table 3: Summary Statistics

Variable	Mean	SD
Panel A: Outcome variables		
RN HPRD	0.427	0.309
LPN HPRD	0.796	0.325
CNA HPRD	2.100	0.548
Total HPRD	3.324	0.793
Panel B: Control variables		
Government (dummy)	0.101	0.301
Non-profit (dummy)	0.171	0.377
Chain affiliation (dummy)	0.546	0.498
Beds	108.4	57.9
Occupancy rate (%)	76.4	15.6
% Medicare	13.0	11.1
% Medicaid	60.6	20.3
Acuity quartile 2 (state-month)	0.231	0.422
Acuity quartile 3 (state-month)	0.230	0.421
Acuity quartile 4 (state-month)	0.233	0.423

Notes: The unit of observation is facility-month. HPRD denotes hours per resident day. RN, LPN, and CNA denote registered nurses, licensed practical nurses, and certified nursing assistants, respectively. Total HPRD is the sum of RN, LPN, and CNA HPRD. Occupancy rate is the ratio of residents to certified beds (percent). % Medicare and % Medicaid are the shares of residents covered by Medicare and Medicaid, respectively. Government and Non-profit are ownership-type indicator variables (for-profit omitted category). Chain affiliation is an indicator for membership in a multi-facility chain. Acuity quartiles are state-month quartiles of resident acuity (case-mix) with quartile 1 omitted. Rows = 632,231; Facilities = 7,806; Treated facilities = 1,589; Period = 2017/01–2024/06; Average months per facility = 81.0.

Table 4 reports summary statistics for the quarterly quality sample. The final sample consists of 7,768 unique skilled nursing facilities and 208,657 facility-quarter observations over the same period as the staffing sample. Of this sample, 1,588 go through an ownership change during the sample period.

Table 4: Summary Statistics

Variable	Mean	SD
Panel A: Selected quality metrics		
Catheter	1.586	2.121
Antipsychotic	14.306	9.287
Hypnotics	20.394	10.269
Pressure injuries	7.793	5.259
Falls with major injury	3.384	2.838
Weight Loss	6.229	4.864
ADL Increase	15.001	8.801
Urinary Tract Infections	2.486	3.237
Panel B: Staffing and control variables		
RN HPRD	0.422	0.299
LPN HPRD	0.795	0.316
CNA HPRD	2.092	0.533
Government (dummy)	0.101	0.301
Non-profit (dummy)	0.170	0.376
Chain affiliation (dummy)	0.549	0.498
Beds	109.0	57.8
Occupancy rate (%)	76.6	15.4
% Medicare	13.3	12.3
% Medicaid	60.7	20.4
Acuity quartile 2 (state-quarter)	0.249	0.433
Acuity quartile 3 (state-quarter)	0.248	0.432
Acuity quartile 4 (state-quarter)	0.252	0.434

Notes: The unit of observation is facility–quarter. RN, LPN, and CNA denote registered nurses, licensed practical nurses, and certified nursing assistants, respectively. HPRD denotes hours per resident day. Occupancy rate is the ratio of residents to certified beds (percent). % Medicare and % Medicaid are the shares of residents covered by Medicare and Medicaid, respectively. Government and Non-profit are ownership-type indicator variables (for-profit omitted category). Chain affiliation is an indicator for membership in a multi-facility chain. Acuity quartiles are state–quarter quartiles of resident acuity (case-mix) with quartile 1 omitted. Rows = 208,657; Facilities = 7,768; Treated facilities = 1,588; Period = 2017Q1–2024Q2; Average quarters per facility = 26.9.

3.2 Staffing Responses to Ownership Change

Figure 1 presents event study estimates of the effect of ownership changes on nursing staffing outcomes (i.e., Equation 2). Following an ownership transition, staffing declines persist for many periods, with the largest and most precise effects occurring for registered nurses (RN), certified nursing assistants (CNA), and total nursing hours per resident day.

RN staffing declines immediately following ownership changes and remains far below pre-

transition levels throughout the post-treatment period. CNA staffing exhibits a similar but slightly smaller decline, while total nursing hours mirror the combined reductions across staff types. In contrast, LPN staffing shows limited and statistically insignificant changes over time. These dynamic patterns indicate that ownership changes are followed by sustained reductions in labor inputs, rather than short-run staffing adjustments that quickly reverse.

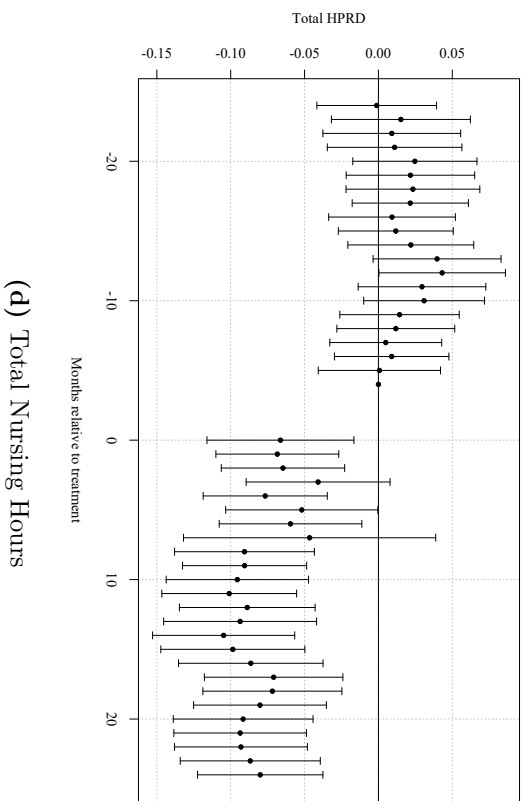
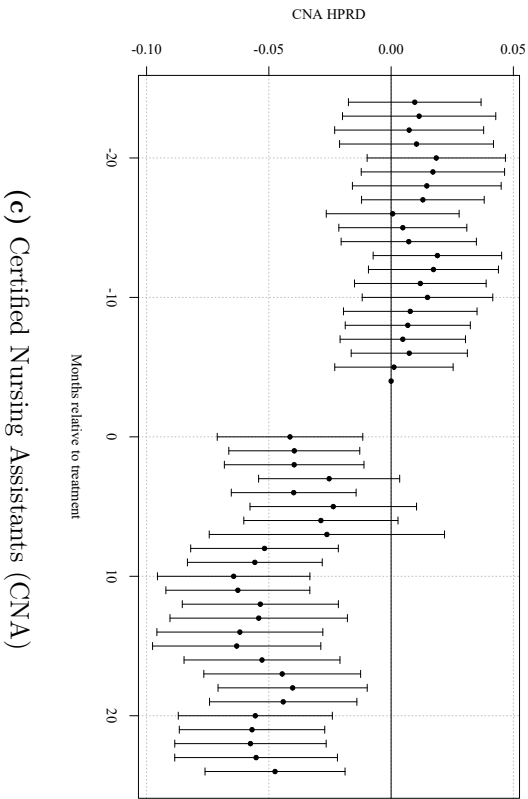
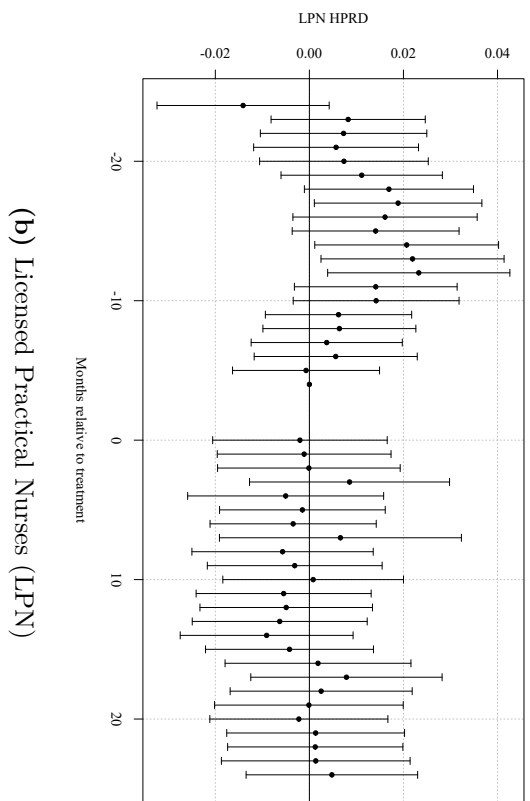
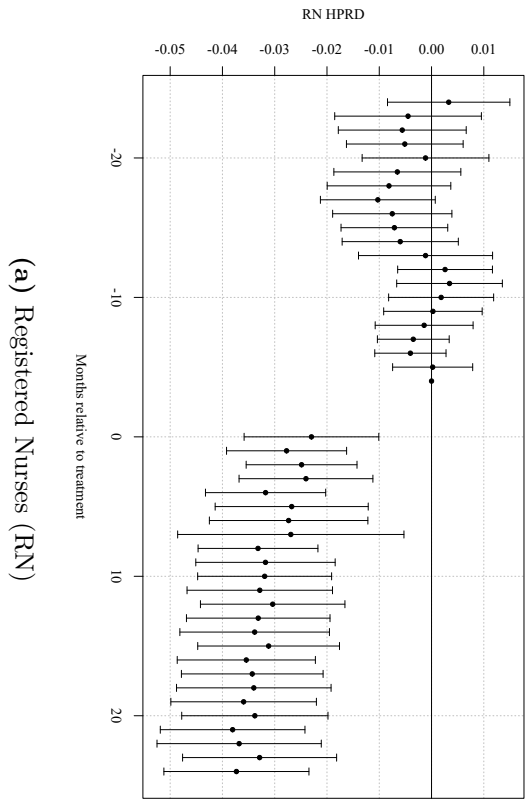


Figure 1: Dynamic Effects of Ownership Change on Nursing Staffing. Event study estimates of the effect of a change in ownership on nursing staff levels in hours per resident day (HPRD). Points show coefficient estimates relative to the reference period $\tau = -4$; vertical bars denote 95% confidence intervals. The three months prior to the ownership change ($\tau = -3, -2, -1$) are excluded. All specifications include control variables and facility and month fixed effects.

To summarize these dynamic effects with a single average treatment estimate, Table 5 reports two-way fixed effects estimates of the ownership change effect (i.e., Equation 4). Panel A reports results in levels, while Panel B reports logged results. Consistent with the event-study evidence, the average post-transition estimates indicate sustained reductions in nursing labor inputs. RN staffing declines by approximately 0.03 hours per resident day (roughly 9 percent), CNA staffing falls by about 0.05 hours (approximately 3 percent), and total staffing decreases by 0.08 hours per resident day (roughly 3 percent). LPN staffing shows no statistically significant change.

Table 5: Two-Way Fixed Effects Estimates of *post* on Staffing Outcomes (Baseline, Without anticipation)

	Outcomes			
	RN	LPN	CNA	Total
HPRD	−0.031*** (0.005)	−0.000 (0.006)	−0.052*** (0.009)	−0.083*** (0.012)
Log(HPRD)	−0.088*** (0.016)	0.003 (0.009)	−0.026*** (0.005)	−0.025*** (0.004)

Notes: Each cell reports the coefficient on *post* with two-way clustered standard errors (by facility and month) in parentheses. The table reports staffing levels (HPRD) and log staffing levels (Log(HPRD)). Sample: Without anticipation ($N_{\text{HPRD}} = 626,311$; $N_{\text{Log}} = 626,311$).

All specifications include facility and month fixed effects and covariates: *government*, *non-profit*, *chain*, *beds*, *occupancy rate*, *percent Medicare*, *percent Medicaid*, and state case-mix quartile indicators.

Statistical significance: *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$.

3.2.1 Heterogeneity Before and After the Onset of COVID-19

Table 6 examines whether these effects differ before and after the COVID-19 pandemic. The estimated reductions are generally smaller during the pandemic period for CNA and total HPRD, while RN reductions remain negative and statistically significant. This pattern suggests that pandemic-era labor market constraints and heightened regulatory scrutiny may have limited some staffing adjustments after ownership change.

I think that Appendix table 8 is interesting since the difference is only statistically significant in LPNs

Table 6: TWFE Estimates of *post*: Pre- vs Post-pandemic Periods (Without anticipation)

	Outcomes			
	RN	LPN	CNA	Total
Panel A: Staffing Levels in HPRD				
Pre-Pandemic Period (2017/01 - 2019/12)	−0.029*** (0.006)	−0.019** (0.008)	−0.063*** (0.014)	−0.111*** (0.019)
Pandemic Period (2020/04 - 2024/06)	−0.018** (0.007)	0.000 (0.008)	−0.041*** (0.015)	−0.058*** (0.020)
Panel B: Log Staffing Levels in HPRD				
Pre-Pandemic Period (2017/01 - 2019/12)	−0.073*** (0.024)	−0.014 (0.012)	−0.032*** (0.007)	−0.034*** (0.006)
Pandemic Period (2020/04 - 2024/06)	−0.065*** (0.021)	0.001 (0.011)	−0.020** (0.008)	−0.019*** (0.006)

Notes: Each cell reports the coefficient on *post* with two-way clustered standard errors (by facility and month) in parentheses. Panel A reports levels (HPRD); Panel B reports logs (HPRD). Sample sizes: Row 1 ($N_{\text{levels}} = 252, 453$; $N_{\text{logs}} = 252, 453$), Row 2 ($N_{\text{levels}} = 356, 174$; $N_{\text{logs}} = 356, 174$).

All specifications include facility and month fixed effects and covariates: *government*, *non-profit*, *chain*, *beds*, *occupancy rate*, *percent Medicare*, *percent Medicaid*, and state case-mix quartile indicators.

Statistical significance: *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$.

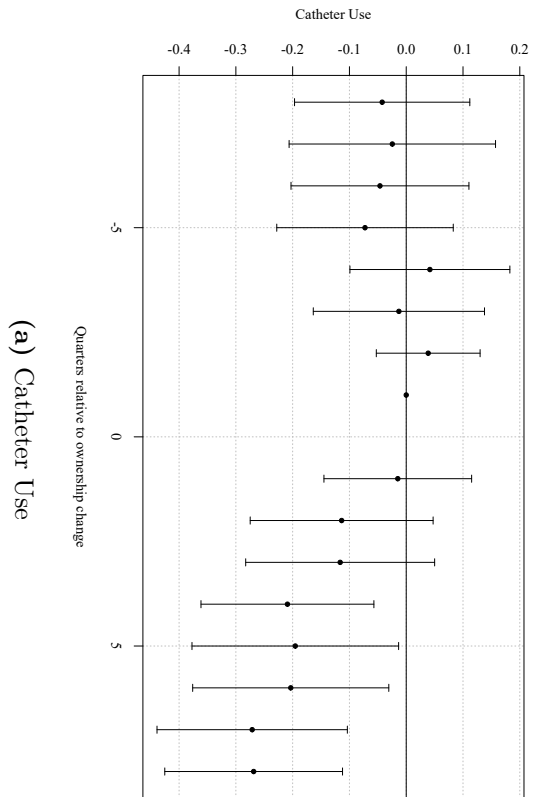
Pre-pandemic 2017/01–2019/12; Pandemic 2020/04–2024/06.

Together, the staffing results show that ownership changes are followed by sustained reductions in measured nursing labor inputs. The declines are concentrated among registered nurses, certified nursing assistants, and total nursing hours per resident day, while licensed practical nurse staffing remains comparatively stable. These patterns suggest that ownership changes are not merely legal or administrative events, but events of operational restructuring in which new owners adjust the mix and intensity of labor used in care production.

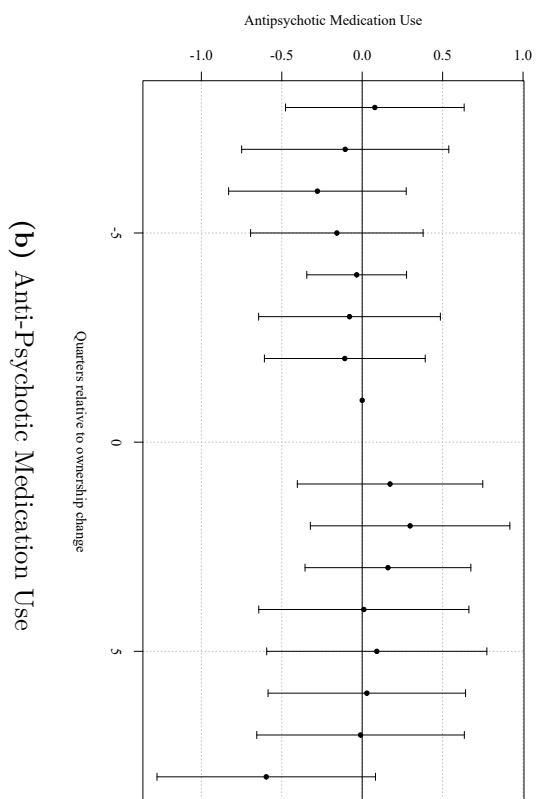
The welfare implications of these input reductions are ambiguous without examining quality outcomes. If staffing reductions primarily reflect harmful cost-cutting, one would expect worsening in resident outcomes, particularly measures that depend heavily on direct staff attention. If, instead, ownership changes disrupt old routines and move facilities closer to their feasible production frontier, reductions in labor inputs may occur alongside improvements in quality measures that are sensitive to managerial routines, monitoring, and care protocols.

3.3 Quality Responses to Ownership Change

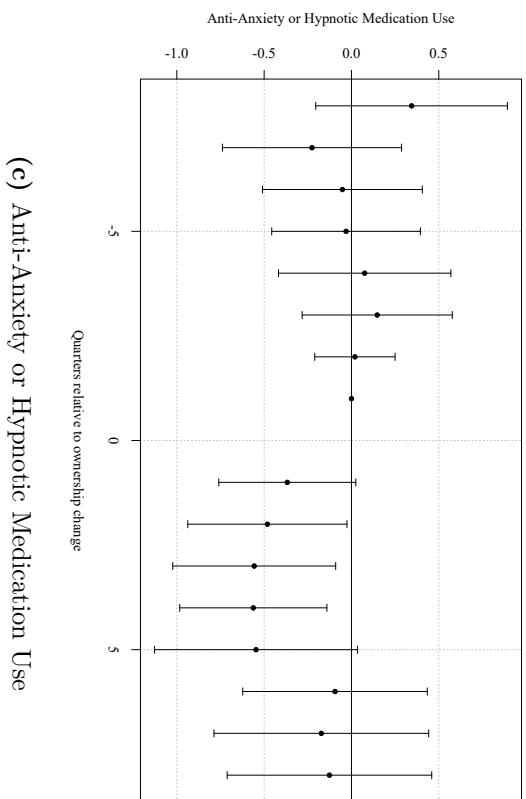
Figure 2 presents event-study estimates for routine-sensitive quality measures. These outcomes are intended to capture changes in care that may respond to changes in oversight, monitoring, documentation, and clinical routines. The estimates show whether ownership changes are followed by changes in quality measures that may be especially responsive to organizational re-optimization.



(a) Catheter Use



(b) Anti-Psychotic Medication Use

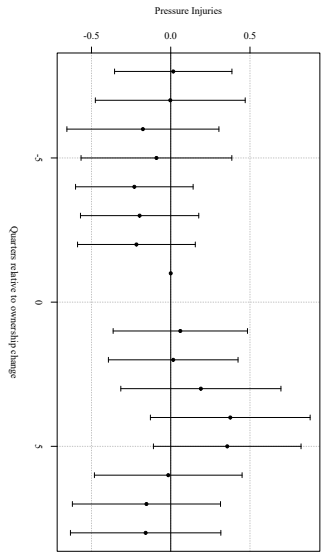


(c) Anti-Anxiety or Hypnotic Medication Use

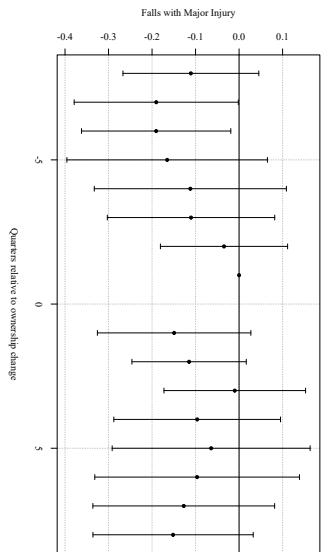
Figure 2: Dynamic Effects of Ownership Change on Routine-Sensitive Process Measures. Event-study estimates of the effect of a change in ownership on routine-sensitive process measures. Points show coefficient estimates relative to the reference period $\tau = -1$; vertical bars denote 95% confidence intervals. The ownership-change quarter ($\tau = 0$) is excluded from the estimation sample. All specifications include control variables and facility and quarter fixed effects.

The results shown in figure 2 provide mixed but generally supportive evidence that ownership changes alter clinical and managerial routines. Catheter use declines following ownership change, and anti-anxiety or hypnotic medication use also falls relative to the reference period. Antipsychotic medication use declines minimally but is not statistically significant. Because lower values indicate better measured quality for all quality outcomes, these patterns suggest that ownership changes are not followed solely by reductions in measured inputs; some quality measures that capture labor saving mechanisms improve after control changes.

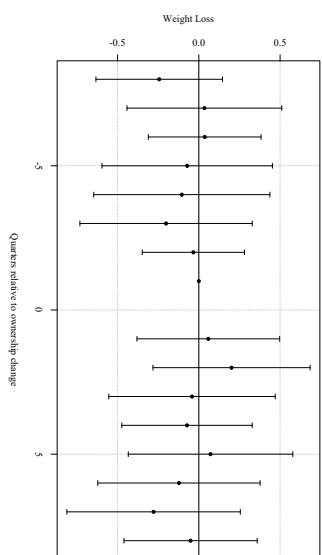
Figure 3 presents event-study estimates for staffing-intensive resident outcomes. These outcomes are more directly tied to hands-on care intensity. If ownership changes primarily reduced inputs in ways that harmed residents, these measures would be the most likely to worsen.



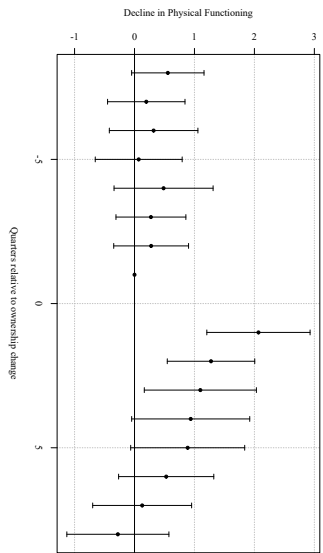
(a) Pressure Injuries



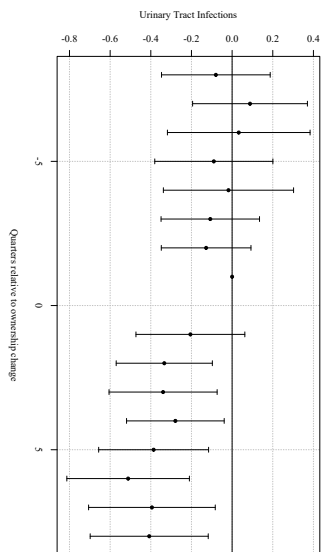
(b) Falls with Major Injury



(c) Weight Loss



(d) Decline in Physical Functioning



(e) Urinary Tract Infections

Figure 3: Dynamic Effects of Ownership Change on Resident Outcome Measures. Event-study estimates of the effect of a change in ownership on resident outcome measures. Points show coefficient estimates relative to the reference period $\tau = -1$; vertical bars denote 95% confidence intervals. The ownership-change quarter ($\tau = 0$) is excluded from the estimation sample. All specifications include control variables and facility and quarter fixed effects.

The resident outcome measures show less evidence of broad worsening. Pressure injuries, falls with major injury, weight loss, and decline in physical functioning do not change in a statistically significant manner. Urinary tract infections do show modest declines. Taken together, these results provide little evidence that staffing reductions, through ownership changes, translate into widespread worsening of the selected resident outcome measures over the observed window.

Table 7 summarizes the quality responses using average post-change TWFE specifications.

Table 7: Two-Way Fixed Effects Estimates of Ownership Change on Quality Measures

Quality Measure	Without Staffing Controls	With Staffing Controls
Routine-Sensitive Process Measures		
Catheter Use	−0.173*** (0.050)	−0.175*** (0.051)
Antipsychotic Medication Use	−0.116 (0.218)	−0.134 (0.218)
Anti-Anxiety or Hypnotic Medication Use	−0.439** (0.211)	−0.421* (0.212)
Resident Outcome Measures		
Pressure Injuries	0.148 (0.179)	0.159 (0.181)
Falls with Major Injury	−0.009 (0.067)	−0.007 (0.067)
Weight Loss	0.102 (0.114)	0.095 (0.113)
Decline in Physical Functioning	0.228 (0.257)	0.254 (0.262)
Urinary Tract Infections	−0.310*** (0.076)	−0.302*** (0.076)

Notes: Each cell reports the coefficient on *Post*, with two-way clustered standard errors by facility and calendar quarter in parentheses.

All specifications include facility fixed effects, year-quarter fixed effects, and time-varying facility controls. The ownership-change quarter, $\tau = 0$, is excluded from the estimation sample.

The first column excludes staffing controls. The second column additionally controls for RN, LPN, and CNA hours per resident day. Because staffing may be affected by ownership change, staffing-controlled estimates are interpreted descriptively rather than as preferred total effects.

Lower values indicate better measured quality for all listed quality measures.

Non-staffing controls: `government + non_profit + chain + beds + occupancy_rate + pct_medicare + pct_medicaid + cm_q_state_2 + cm_q_state_3 + cm_q_state_4`.

Staffing-controlled specification: `government + non_profit + chain + beds + occupancy_rate + pct_medicare + pct_medicaid + cm_q_state_2 + cm_q_state_3 + cm_q_state_4 + rn_hprd + lpn_hprd + cna_hprd`.

Outcome-specific windows and estimation sample sizes: Catheter Use [2017Q1–2024Q2; N=197,438]; Antipsychotic Medication Use [2017Q1–2024Q2; N=195,998]; Anti-Anxiety or Hypnotic Medication Use [2017Q1–2024Q2; N=196,555]; Pressure Injuries [2018Q1–2023Q3; N=139,764]; Falls with Major Injury [2017Q1–2024Q2; N=200,132]; Weight Loss [2017Q1–2024Q2; N=195,249]; Decline in Physical Functioning [2017Q1–2024Q2; N=187,542]; Urinary Tract Infections [2017Q1–2024Q2; N=199,311].

Statistical significance: *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$.

The estimates are similar with and without staffing controls. Because staffing is itself affected by ownership change, the staffing-controlled specifications are not interpreted as preferred total effects. Instead, their similarity to the baseline estimates suggests that the

measured quality responses are not mechanically explained by contemporaneous changes in RN, LPN, and CNA hours per resident day.

3.4 Robustness

This section reports robustness checks for our staffing results, which are the main input-side estimates in our paper. These checks are designed to assess the validity of our identifying assumptions and the sensitivity of our results to alternative model choices. Across all exercises, the estimated effects of ownership change on staffing remain stable in sign, magnitude, and statistical significance. The supporting tables and figures are reported in the appendix.

3.4.1 Alternative Comparison Groups

Our main event-study specification is estimated using two-way fixed effects (TWFE) with staggered treatment timing. In this setting, TWFE can implicitly use already-treated facilities as controls for cohorts treated later, which may distort event-study estimates when treatment effects are dynamic or heterogeneous across cohorts. To assess sensitivity to this issue, we re-estimate the event study using a stacked difference-in-differences design.

In the stacked approach, we construct cohort-specific samples for each treatment month g . For a given cohort, treated facilities are those with an ownership change in month g , and the control group consists only of facilities that are never treated or not-yet-treated relative to g (i.e., with treatment dates after g). We restrict each cohort-specific sample to a symmetric event window around g and then stack these cohort datasets into a single analysis file. Estimation includes the same covariates as the baseline specification and fixed effects that absorb facility-level time-invariant heterogeneity and common calendar shocks; standard errors are clustered at the facility level.

Appendix Figures 4a through 4d show that the stacked event-study estimates closely track the baseline TWFE event-study results: the post-transition dynamics and the relative magnitudes across staffing types remain qualitatively unchanged. In addition, Appendix

I polished this section up, but refrained from adding new robustness until we talked about it

Table ?? reports joint Wald tests of pre-treatment coefficients in the stacked specification (under the donut design used in the baseline), which are consistent with flat pre-trends in the remaining pre-period. Together, these results suggest that the main findings are not driven by treated-as-control comparisons that can arise in TWFE under staggered adoption.

3.4.2 Event Window and “Donut”

As we have previously discussed, because of changes to staffing in anticipation of a formal ownership change, the assumptions of parallel trends is violated. As a result of this, in our baseline specifications we “donut” equation 2 and equation 4 by removing $\tau \in \{-3, -2, -1\}$. To test the validity of removing this set of pre-treatment coefficients, we estimate equations 2 and 4 with different “donut” sets and show that the estimated post-treatment effects remain largely unchanged. Row 6 of Table 10 reports estimates of equation 4 with a wider anticipatory period excluded from the sample. The results from row 6 show no differences in values or statistical significance. Similarly, row 7 reports estimates under a smaller anticipatory window that is excluded from the data, and estimates remain similar in magnitude and statistical significance.

3.4.3 Gap Size

Because staffing data is occasionally missing or reported with gaps, we examine whether our results are sensitive to the length of time between consecutive observations. Restricting the sample to facilities with smaller reporting gap shows that estimates are similar to those in the full sample, indicating that data gaps do not drive our findings. Row 2 of Table 10 reports our results for equation 4 with the restriction that facilities with a reporting gap of larger than 6 are excluded from our sample. Compared to the baseline (row 1) this result is identical across staff types and statistical significance. The exact same result is true for row 3 which reports estimates for equation 4 where facilities with a reporting gap of greater than 3 are removed. Row 4 and 5 report estimates where gap greater than 1 or equal to 0

respectively, are removed. Similarly, we see no differences in these results with respect to the baseline.

4 Discussion

This paper studies ownership changes in nursing homes as moments of organizational disruption in care production. Our results show that ownership transitions are followed by sustained reductions in measured nursing labor inputs, especially RN hours, CNA hours, and total nursing hours per resident day. At the same time, we find that some selected quality measures improve: catheter use and anti-anxiety or hypnotic medication use decline, and urinary tract infections also fall. Other resident outcome measures, including pressure injuries, falls with major injury, weight loss, and decline in activities of daily living, show little evidence of broad worsening. Taken together, these patterns suggest that ownership changes do more than alter the legal identity of the owner. They appear to change how facilities organize production.

The combination of lower staffing inputs and selected quality improvements is consistent with the idea that some facilities operate inside their feasible production frontier before undergoing ownership change. New owners do not necessarily possess a superior care technology. Instead, the ownership transition may disrupt old routines, alter managerial oversight, change medication-management practices, or reduce organizational slack. If facilities, under previous ownership, relied on inefficient routines, then new owners could reduce some inputs while maintaining or improving selected measured quality outcomes. This is consistent with the idea that ownership change may not be a mechanism for cost-containment, but rather a way for new owners to re-optimize production inputs.

4.1 Interpreting Staffing Reductions

The results of our staffing specifications indicate that both the level and mix of nurse staffing adjust following a change in ownership. The declines are concentrated among RNs and CNAs, while LPN staffing remains comparatively stable. In a regulated sector with limited short-run pricing flexibility, staffing is one of the most immediately adjustable cost-saving mechanisms available to new owners.

The relative stability of LPN staffing is informative because LPNs occupy an intermediate position in nursing home production. RNs perform higher-skill clinical and supervisory functions, while CNAs provide much of the hands-on daily care ([Lin, 2014](#)). LPNs can perform routine clinical tasks and help maintain day-to-day coverage, making them a potential margin of adjustment when facilities reduce RN and CNA hours. The absence of a comparable decline in LPN staffing is therefore consistent with compositional adjustment rather than uniform staffing cuts across all nurse types.

An important question that can be raised is whether the estimated staffing reductions are economically large. Benchmarking the estimates against baseline staffing levels helps provide context. In our sample, mean staffing levels are 0.427 RN HPRD, 0.796 LPN HPRD, 2.100 CNA HPRD, and 3.324 total HPRD. In our average post-treatment effect specification, ownership change reduces RN staffing by approximately 0.03 HPRD, CNA staffing by approximately 0.05 HPRD, and total staffing by approximately 0.08 HPRD. Relative to baseline staffing levels, these estimates imply declines of roughly 7 to 9 percent for RN staffing and about 3 percent for CNA and total staffing. Expressed in minutes, the total staffing reduction is approximately five fewer nursing minutes per resident-day. These changes may be modest and should not, by themselves, be seen as evidence of substantial welfare losses. However, staffing is a core input into care production, so even small reductions may matter in facilities operating close to staffing constraints or for residents requiring more intensive assistance.

4.2 Quality Responses and the Reorganization of Care

The results of our regressions on quality help distinguish between alternative interpretations of the staffing reductions. If ownership changes primarily reflected quality-reducing cost containment, we would expect staffing-intensive resident outcomes to become worse after the ownership change. The evidence does not show broad worsening in the selected resident outcome measures. Pressure injuries, falls with major injury, weight loss, and decline in physical functioning are not statistically distinguishable from zero in the average post-change specifications. Urinary tract infections decline.

At the same time, we find that selected routine-sensitive process measures improve. Catheter use falls, and anti-anxiety or hypnotic medication use declines after ownership change. Anti-psychotic medication use also declines moderately, though the average post-change estimate is not statistically significant. These measures may respond to changes in care routines, medication management, monitoring, documentation, and managerial oversight. Their improvement suggests that ownership changes may lead facilities to revise care processes rather than simply reduce labor inputs.

This pattern is consistent with incumbent nursing home owners operating their facilities inside their feasible production frontier. A facility can be inside its feasible production frontier if existing routines persist even when better combinations of inputs and outcomes are attainable. Ownership change may interrupt those routines by introducing new managers, new monitoring systems, different performance targets, or greater willingness to change staffing and care practices. Under this interpretation, lower RN and CNA hours do not necessarily imply a proportional decline in care quality. Instead, they may reflect a reorganization of production in which some inefficient practices are reduced while selected quality measures improve.

I was thinking about putting a paragraph about how the quality metrics are limited and don't capture all aspects of resident welfare. Therefore, our results should be interpreted very specifically. Thoughts?

4.3 Heterogeneity and Constraints

The heterogeneity results of our additional analyses provide insight into the constraints facing new owners. Staffing reductions are smaller during the COVID-19 period for several outcomes, suggesting that pandemic-era labor market tightness, heightened scrutiny, and operational disruption may have limited owners' ability to adjust staffing. When labor markets are tight or regulatory attention is elevated, the feasible set of staffing reductions may narrow.

Differences by organizational structure may also matter. Chain-affiliated facilities may have access to internal labor markets, standardized management practices, and administrative support that allow them to absorb ownership changes with smaller staffing disruptions. Independent facilities may face tighter financial or managerial constraints, making staffing a more immediate adjustment margin. These patterns are consistent with the broader interpretation that ownership changes operate through internal production decisions and that the scope for adjustment depends on capacity and constraints.

Keep?

4.4 Limitations

Several limitations should be considered when interpreting our results. First, despite our two-step approach to identifying ownership transitions, the timing of ownership change may still be measured with error. The recorded ownership change date may not capture earlier contract negotiations, transition planning, or operational changes that occur before the formal closing date. Our preferred staffing specification addresses this concern by excluding the immediate pre-transition months, and our preferred quality specification excludes the ownership-change quarter, but some timing error may remain.

Second, ownership changes are not randomly assigned. The empirical design helps address time-invariant facility differences and common shocks, but ownership changes may still coincide with unobserved facility-level shocks such as financial distress, management turnover, or local labor market changes. These shocks could affect both staffing and quality.

Third, the staffing and quality measures are imperfect. PBJ staffing data may contain reporting gaps or measurement error, although robustness checks suggest that reporting gaps do not drive the main staffing results. CMS quality measures are also limited. They capture selected process and resident outcome measures but do not fully measure resident welfare, resident experience, deficiencies, hospitalizations, mortality, or other aspects of quality.

Finally, our results should not be interpreted as proving that ownership changes improve welfare. The evidence shows that ownership changes reduce selected labor inputs while some quality measures improve and others do not worsen. This pattern is consistent with routine replacement and inside-frontier inefficiency, but it is also possible that unmeasured aspects of care worsen or that effects differ across resident types and facilities. Future work should examine whether these ownership changes affect broader outcomes and whether the observed input reductions are concentrated in facilities with greater pre-transition slack.

5 Conclusion

Ownership changes are common in U.S. nursing homes, yet their implications for care production remain poorly understood. This paper studies ownership transitions as moments of organizational disruption and reoptimization. Using CMS administrative panel data from 2017 through 2024, we identify ownership transitions that reflect changes in economic control and estimate their effects on staffing inputs and selected quality outcomes. We find that ownership changes are followed by sustained reductions in RN staffing, CNA staffing, and total nursing hours per resident day, while LPN staffing remains comparatively stable. These staffing responses indicate that ownership transitions alter the internal organization of care production rather than merely changing the legal identity of the facility owner.

Our results focused on quality complicate a simple cost-cutting interpretation. If ownership changes reduced staffing in ways that broadly harmed residents, we would expect worsening in staffing-intensive resident outcomes. Instead, selected routine-sensitive process

measures improve, including catheter use and anti-anxiety or hypnotic medication use, while urinary tract infections also decline. Other resident outcome measures, including pressure injuries, falls with major injury, weight loss, and decline in physical functioning, show little evidence of broad worsening over the observed post-transition window. These patterns suggest that input reductions after ownership change do not translate mechanically into measured quality declines.

Our findings are consistent with ownership transitions revealing inside-frontier inefficiency in nursing home production. New owners do not necessarily possess a superior care technology, but they may be more willing to revise old routines, alter staffing composition, reduce organizational slack, and change medication-management or monitoring practices. In this sense, ownership change may operate as a disruption that shifts facilities toward a more efficient input-quality combination. At the same time, the welfare implications remain ambiguous. Our analysis does not directly estimate a production frontier, and the quality measures used do not capture all dimensions of resident welfare. Unmeasured outcomes, resident experience, deficiencies, hospitalizations, and mortality may respond differently.

This paper contributes to the literature by shifting the focus from ownership changes as solely market-structure events to ownership changes as shocks to the internal organization of care. In nursing homes, where prices are heavily regulated and staffing is a central input, changes in control can reshape production even when the facility remains in the same market and continues serving the same local population. Future work should examine which types of ownership changes generate efficiency-enhancing reorganization, which produce quality-reducing cost containment, and whether the observed input reductions are concentrated in facilities with greater pre-transition organizational slack.

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Tables

Table 8: TWFE Post Estimates with Interaction: Pre-pandemic vs Pandemic (Without anticipation)

	Outcomes			
	RN	LPN	CNA	Total
Panel A: Staffing Levels in HPPD				
Pre-pandemic: <i>post</i>	−0.023*** (0.006)	−0.016** (0.008)	−0.040*** (0.012)	−0.079*** (0.017)
Difference (Pandemic − Pre): <i>post</i> × Pandemic	−0.010* (0.006)	0.019*** (0.007)	−0.017 (0.011)	−0.009 (0.015)
Pandemic effect: <i>post</i> + interaction	−0.033*** (0.005)	0.002 (0.006)	−0.057*** (0.010)	−0.088*** (0.013)
Panel B: Log Staffing Levels				
Pre-pandemic: <i>post</i>	−0.076*** (0.024)	−0.021* (0.012)	−0.014** (0.007)	−0.022*** (0.005)
Difference (Pandemic − Pre): <i>post</i> × Pandemic	−0.020 (0.022)	0.027*** (0.010)	−0.016** (0.006)	−0.005 (0.005)
Pandemic effect: <i>post</i> + interaction	−0.096*** (0.017)	0.006 (0.009)	−0.030*** (0.005)	−0.027*** (0.004)

Notes: Each cell reports the indicated coefficient with two-way clustered standard errors (by facility and month) in parentheses.

All specifications include facility and month fixed effects and covariates: *government*, *non-profit*, *chain*, *beds*, *occupancy rate*, *percent Medicare*, *percent Medicaid*, and state case-mix quartile indicators.

Sample: 2017/01–2019/12 and 2020/04–2024/06 (excluding 2020/01–2020/03), without anticipation ($N = 610,364$).

Statistical significance: *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$.

Table 9: TWFE DiD Estimates of *post* on Staffing Outcomes (Stacked Sample, Baseline Donut)

	Outcomes			
	RN	LPN	CNA	Total
HPPD	−0.023*** (0.004)	−0.007 (0.005)	−0.051*** (0.008)	−0.082*** (0.011)
Log(HPPD)	−0.066*** (0.014)	−0.003 (0.008)	−0.027*** (0.004)	−0.026*** (0.003)

Notes: Each cell reports the coefficient on *post* with facility-clustered standard errors in parentheses. The stacked sample includes never-treated and not-yet-treated controls for each cohort; the donut excludes $\tau \in \{-3, -2, -1\}$. Sample sizes: $N_{\text{HPPD}} = 23,877,687$; $N_{\text{Log}} = 23,492,034$.

Specifications include facility-by-cohort fixed effects (*stack_id*), calendar-month fixed effects, cohort fixed effects, and covariates: *government*, *non-profit*, *chain*, *beds*, *occupancy rate*, *percent Medicare*, *percent Medicaid*, and state case-mix quartile indicators.

Statistical significance: *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$.

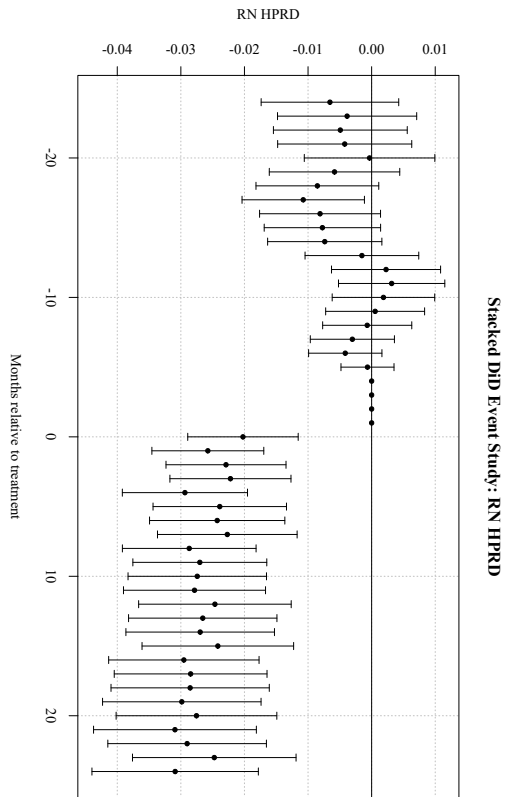
Table 10: TWFE estimates of *post* (without anticipation).

		Outcomes			
	N	RN	LPN	CNA	Total
Panel A: Staffing Levels in HPPD					
(1) Baseline	628,107	−0.032*** (0.005)	0.000 (0.006)	−0.054*** (0.009)	−0.086*** (0.013)
(2) Sample excludes <i>gap</i> > 6	627,923	−0.032*** (0.005)	−0.000 (0.006)	−0.054*** (0.009)	−0.086*** (0.013)
(3) Sample excludes <i>gap</i> > 3	627,626	−0.032*** (0.005)	−0.000 (0.006)	−0.054*** (0.009)	−0.086*** (0.013)
(4) Sample excludes <i>gap</i> > 1	623,585	−0.032*** (0.005)	0.000 (0.006)	−0.055*** (0.009)	−0.086*** (0.013)
(5) Sample excludes <i>gap</i> > 0	623,585	−0.032*** (0.005)	0.000 (0.006)	−0.055*** (0.009)	−0.086*** (0.013)
(6) Drop $t \in \{-4, -3, -2, -1\}$	626,729	−0.032*** (0.005)	0.000 (0.006)	−0.055*** (0.009)	−0.086*** (0.013)
(7) Drop $t \in \{-2, -1\}$ only	628,107	−0.032*** (0.005)	0.000 (0.006)	−0.054*** (0.009)	−0.086*** (0.013)
Panel B: Log Staffing Levels in HPPD					
(1) Baseline	628,107	−0.092*** (0.016)	0.003 (0.009)	−0.028*** (0.005)	−0.026*** (0.004)
(2) Sample excludes <i>gap</i> > 6	627,923	−0.092*** (0.016)	0.003 (0.009)	−0.028*** (0.005)	−0.026*** (0.004)
(3) Sample excludes <i>gap</i> > 3	627,626	−0.092*** (0.016)	0.003 (0.009)	−0.028*** (0.005)	−0.026*** (0.004)
(4) Sample excludes <i>gap</i> > 1	623,585	−0.092*** (0.016)	0.004 (0.009)	−0.028*** (0.005)	−0.026*** (0.004)
(5) Sample excludes <i>gap</i> > 0	623,585	−0.092*** (0.016)	0.004 (0.009)	−0.028*** (0.005)	−0.026*** (0.004)
(6) Drop $t \in \{-4, -3, -2, -1\}$	626,729	−0.092*** (0.016)	0.003 (0.009)	−0.028*** (0.005)	−0.026*** (0.004)
(7) Drop $t \in \{-2, -1\}$ only	628,107	−0.092*** (0.016)	0.003 (0.009)	−0.028*** (0.005)	−0.026*** (0.004)

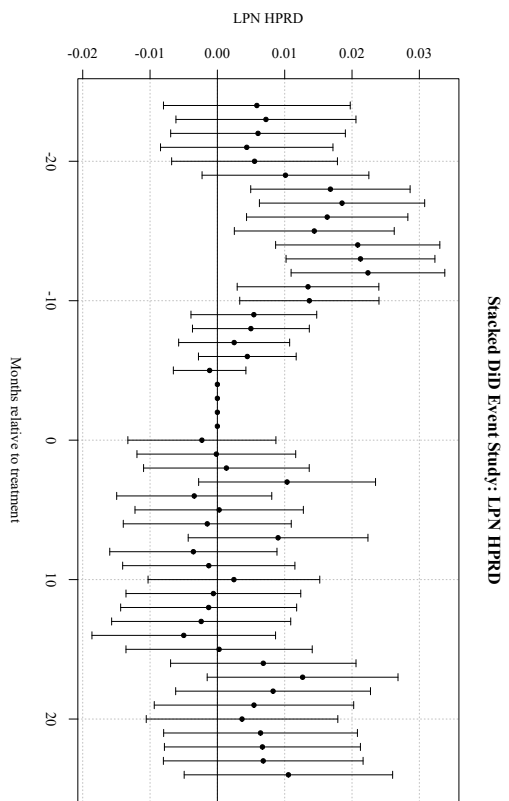
Notes: Each cell reports the *post* coefficient with two-way clustered standard errors at the facility and month levels. Panel A uses staffing levels (HPPD); Panel B uses logs of HPPD.

All specifications include facility and month fixed effects and controls for ownership (government, non-profit, chain), beds, occupancy rate, payer mix (Significance: *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$).

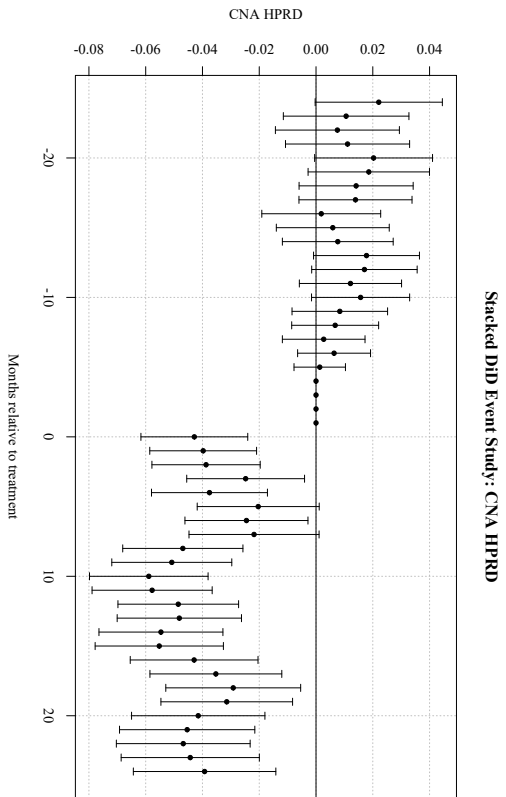
Figures



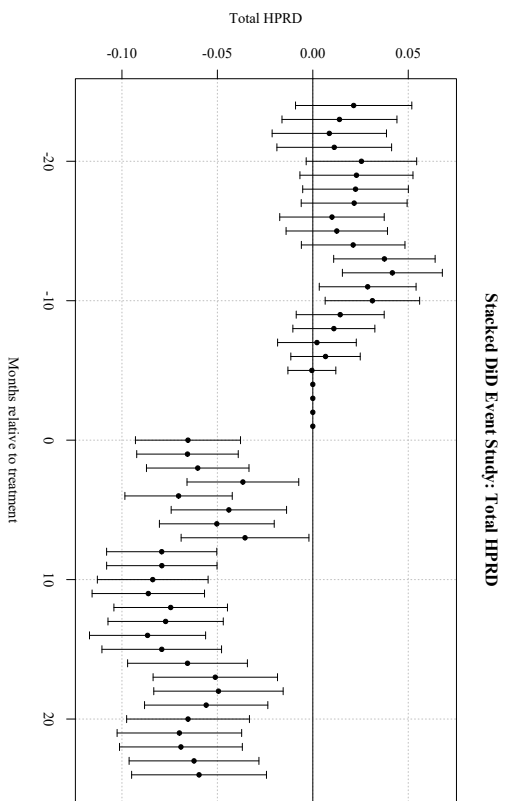
(a) Registered Nurses (RN)



(b) Licensed Practical Nurses (LPN)



(c) Certified Nursing Assistants (CNA)



(d) Total Nursing Hours

Figure 4: Stacked Event-Study Estimates of Ownership Change on Nursing Staffing. The stacked design compares each treated cohort only to never-treated and not-yet-treated facilities within the event window, preventing already-treated facilities from serving as controls. The donut excludes $\tau \in \{-3, -2, -1\}$; the reference period is $\tau = -4$.